



A care companion for older people and patients with chronic conditions. Patients choose what to share with family members and carers; their choices appear in the GP consent view.

Eleanor Chen, 83

SIM-000006

- Four abnormal blood panels since May, white cells **2.7 · 1.6 · 1.3 · 2.1** $\times 10^9/L$ against a range of 4 to 11. **Each one was filed. Nobody looked at them together.** The only message she ever received was about paperwork.
- Admitted at **04:20 this morning**. A GP appointment still booked for 08:30.
- Landline only. Her husband could not drive her to her last review. **Nothing in her record has gone to her family.**

She is the human middleware between her clinicians and her family.

"I want to know what that is. I want to know what's going on. I want to be aware."

This is not one patient.

47%

of blood results have **no record of being communicated** to the patient. 31% for abnormal results.

PACT audit, 2,572 patients, 57 practices

5.6m

results a month, around **68m a year**, with no evidence the patient was told.

EXTRAPOLATED 12m NHS App result views a month at 47%

35,000

GP-hours every working day spent managing results.

EXTRAPOLATED 74 minutes a day per doctor across 28,426 FTE GPs in England

1reading

is not effective at spotting decline. **The trajectory is**, and every GP system already holds the data.

41 serious incidents in 13 months from uncommunicated results, including a 45 mm lung mass (HSIB 2019). 5 million unpaid carers. 0.4% of portal users have shared access.

Results go nowhere, then aren't understood, then the family is locked out.

01

Not seen coming

Results are reviewed one at a time. No one reads the trend.

02

Not told

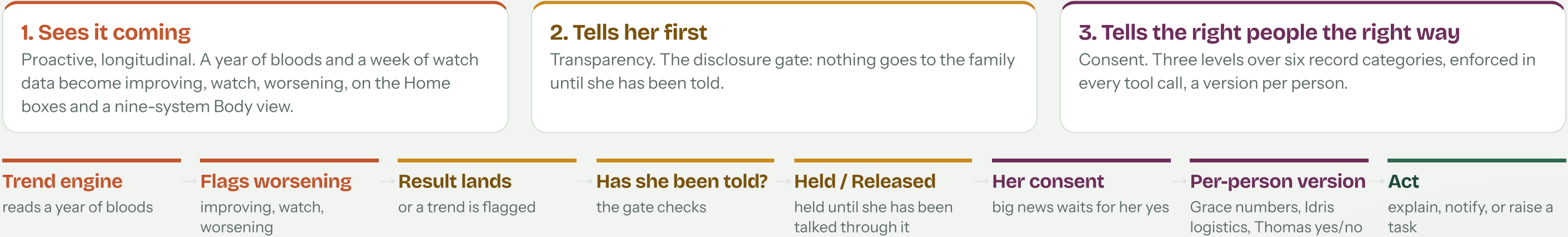
No reliable “patient informed” state exists in the record, and nothing routes a result to the people who need to act.

🔒 03

Not understood, locked out

The App shows the number and says ring your GP; only 51% of adults can find the out-of-range value. Proxy access hands a person the whole record after a practice-led setup, and almost nobody has it.

Kindred turns every result into the right explanation and next action for every trusted person, under the patient's control.



90 seconds, live against Anima.

- **Eleanor's body.** Nine systems coloured from her live record in the Body tab, consent-gated per system; the Home boxes say what needs watching. Trend cards

LIVE FROM THE PROTOTYPE
- **Order the LFT in Anima; advance the clock 121 minutes.** The result lands as a real state change.
- **Held.** Eleanor's phone: a result has arrived and the practice will talk her through it. Family sees nothing.
- **Her yes.** Three versions go out: Grace numbers, Idris logistics, Thomas yes/no. A task appears in Anima's activity trail.
- **Grace asks about a result she cannot see.** NOT_SHARED, no hint. She asks for access; Eleanor approves from a card; the GP consent view updates live.
- **The morning check** reads the 08:30 appointment from the record and posts to the family group: who can give Eleanor a lift?

“There are 3 shifts, but only one core purpose: to put power in patients' hands.”

Chapter 1, p22

“Patients will have control over whether this data is shared.”

Chapter 3, My Health, pp50 to 51. And My Carer, p51: carers gain access to the App on their loved one's behalf.

“The most common reasons for complaints is poor communication.”

Chapter 6, p89

Chapters 2, 3 and 6.

No result ever sits uncommunicated.

And when it is communicated, every trusted person gets the right version, in their words, under her control.

3 × 6

sharing levels over six record categories, plus custom mixes

Every

agent tool call checks consent and is written to the audit rail

9

body systems coloured from the live record, consent-gated per system, on a licensed 3D figure

1

transaction mirrors each consent change to the GP view: observation plus audit

Next: fold the disclosure gate, the escalation and the 30-day trend rules, prototyped today against the live Anima world, into the app's scheduled check. Pilot at one practice. Wire consent to NHS App proxy access.